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PROLONGED FIELD CARE

PHASE 1: PREPARE FOR PFC
PHASE 2: MAINTENANCE
PHASE 3: PREPARE FOR EVACUATION

PREPARE FOR PROLONGED FIELD CARE

CREATE A SAFE ENVIRONMENT TO PERFORM MAINTENANCE PHASE IF REQUIRED
CONTINUE RESUSCITATION AND REASSESSMENT FOR CRITICAL INTERVENTIONS
SECONDARY SURVEY IF FEASIBLE
PRIORITIES OF CARE IDENTIFIED AND RELAYED BACK TO MEDEVAC TEAM

MAINTENANCE (5 PHASES)

MONITOR AND RESUSCITATE

FULL MONITORING AND RESUSCITATE FROM PRIORITIES OF CARE IDENTIFIED EARLIER
REQUEST FOR SPECIALIST CARE OR REACHBACK IF REQUIRED
BLOOD PRODUCTS IF POSSIBLE OR CONSIDER EDP IF FEASIBLE

PAIN RELIEF AND MEDICATIONS

ANALGESIA REGIMEN (CONSIDER MULTIMODAL APPROACHES TO REDUCE OPIOID USE)
ANTIBIOTICS AS EARLY AS POSSIBLE
ANTIFIBRINOLYTICS FOR CLOTTING (2ND DOSE OF TXA)

WOUND CARE AND INFECTION CONTROL

DOWNGRADE TQ's IF FEASIBLE, PACK WOUNDS APPROPRIATELY AND USE STERILE DRESSINGS
CONSIDER LOCAL ANAESTHETIC FOR WOUND MANAGEMENT
REDUCE THE RISK OF SEPSIS - REGULAR MONITORING OF PATIENT TO IDENTIFY DETERIORATION

PATIENT CARE

HYDRATION - ORAL OR INTRAVENOUS FLUIDS AND MONITOR USING URINARY OUTPUT (0.5ml/kg/hr)
HYGIENE - CONSIDER TOILETING ISSUES AND CLEAN PATIENT TO AVOID SORES/INFECTION
NUTRITION - REGULAR FOOD IF ABLE TO EAT, MONITOR GLUCOSE

DOCUMENTATION

[20201105_FMed_1092 V1.1.pdf \(mod.uk\)](#)
CASUALTY ASSESSMENT CHARTS WITH NEWS MONITORING

PREPARE FOR EVACUATION

PATIENT PACKAGED FOR EVACUATION
ALL TUBES AND LINES SECURED
HEARING AND EYE PROTECTION FOR THE PATIENT
BAGS PACKED FOR TRANSFER
RESUPPLY INBOUND
PAPERWORK READY FOR HANDOVER
PATIENT STABLE FOR TRANSFER

KIT MANAGEMENT - THINK RESOURCES, LOCAL SUPPLIES
TEAM MANAGEMENT - TRAIN TEAM MEDICS TO MONITOR